

NIC CPD Micro-Credential: First Aid & Basic Life Support for Professional Caregivers

Issuing organisation: National Institute of Caregivers Nigeria (NIC)

Credential type: Stand-alone Continuing Professional Development (CPD) Micro-Credential

Online learning duration: 5–6 CPD learning hours

Practical component: Recommended and separately assessed where the credential is intended to represent hands-on competence

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Critical credential distinction: Completion of online learning and assessment demonstrates knowledge and scenario judgement. It does **not**, by itself, prove physical competence in CPR, rescue breathing, AED use, choking response or first-aid procedures. A practical competency endorsement may be issued only after successful hands-on assessment by an appropriately qualified instructor or approved assessor.

Scope statement: This is caregiver CPD. It is not a regulated clinical qualification, nursing qualification, statutory workplace certification, professional licence or substitute for local emergency training. Learners must follow current recognised guidance, workplace procedures, equipment instructions and emergency services direction.

1. Course title

NIC CPD Micro-Credential: First Aid & Basic Life Support for Professional Caregivers

2. Course overview

This CPD prepares professional caregivers to recognise common emergencies, make the scene safer, activate appropriate help, provide caregiver-level first aid and basic life support within training, use available emergency equipment appropriately, communicate clearly and document the event.

First aid is immediate assistance for illness or injury. Basic life support (BLS) focuses particularly on recognition of life-threatening emergencies and immediate resuscitation support, including CPR and AED use. The two overlap: a caregiver may provide first aid for a

burn, bleeding or seizure and may provide BLS when a person is unresponsive and not breathing normally.

The course uses current recognised guidance from the 2025 American Heart Association CPR and Emergency Cardiovascular Care guidelines, International Liaison Committee on Resuscitation publications and the International Federation of Red Cross and Red Crescent Societies first-aid guidance [1](#) [2](#) [3](#) . NIC should verify the applicable guidance version before launch and update technical content when the recognised authority changes its recommendations.

The online credential must be separated from practical skills verification. CPR, rescue breaths, AED operation, choking response and physical first-aid techniques require supervised practice and assessment.

Course at a glance

Element	Specification
Audience	Professional caregivers, healthcare assistants, home-care workers, residential-care staff, community-care workers and supervisors
Online duration	5–6 CPD learning hours
Structure	6 modules, 18 lessons, module assessments and final assessment
Central sequence	RECOGNISE → ENSURE SAFETY → RESPOND → CALL FOR HELP → PROVIDE FIRST AID/BLS → MONITOR → HAND OVER → DOCUMENT
Online pass mark	80% (28/35) recommended
Practical endorsement	Separate supervised skills verification only
Jurisdictional limitation	Not statutory certification, licensure or legal authorisation unless independently verified

3. Strategic purpose

Caregivers may be the first person present when a client collapses, chokes, bleeds, falls, has a seizure, develops breathing difficulty or experiences another emergency. Early recognition, scene safety, emergency activation and appropriate action can reduce avoidable delay.

The CPD is designed to help a working caregiver answer: **What is dangerous? Is the person responsive and breathing normally? What immediate help is needed? What can I safely do? Who must be called? What must I hand over and document?**

The course does not encourage improvisation, diagnosis or treatment beyond training. It teaches learners to avoid becoming the second casualty, activate help early and use the person's emergency plan and local procedures.

4. Target audience

The target audience is professional caregivers, healthcare assistants, home-care workers, residential-care staff, community-care workers, care supervisors and other personnel who may be the first person present when a client becomes suddenly unwell or injured.

The course is suitable for learners working in homes, facilities, community services and healthcare environments. Nigerian emergency infrastructure varies between locations; therefore, employers must insert current facility contacts, local emergency numbers and backup communication routes before release.

5. Recommended CPD hours

The online learning component should be awarded **5–6 CPD learning hours**. Practical skills verification should be scheduled separately and should not be represented as completed merely because the learner viewed an online demonstration.

6. Prerequisites

Learners should understand basic caregiving, professional boundaries, observation, communication, incident reporting and escalation. No clinical qualification is required for the online component, but practical verification requires a suitable training environment, manikins, AED trainer, first-aid equipment and an appropriately qualified instructor or assessor.

7. Learning outcomes

By the end of the online CPD, the learner should be able to:

1. Distinguish first aid from basic life support.
2. Make an emergency scene safer and avoid becoming a second casualty.
3. Recognise unresponsiveness, absent or abnormal breathing, severe bleeding, choking and other urgent conditions.

4. Activate facility, supervisor, clinical and emergency assistance with useful location information.
5. Explain the caregiver-level adult CPR and AED sequence according to current recognised guidance.
6. Explain choking response, severe bleeding control and shock awareness.
7. Recognise and respond appropriately to fainting, collapse, seizures, burns, falls, head injury and poisoning.
8. Recognise anaphylaxis, breathing difficulty and diabetic emergencies and escalate appropriately.
9. Identify caregiver actions for children, infants, older adults and people with disability without pretending that one protocol fits all.
10. Communicate clearly during emergency handover.
11. Document and report emergencies, incidents and near misses.
12. Demonstrate online knowledge separately from hands-on practical competence.

8. Core competency

Recognise common emergencies, respond promptly and safely within caregiver scope, provide appropriate first aid and basic life support, activate emergency assistance, use available emergency equipment appropriately, and communicate and document the event according to organisational procedures.

9. Emergency response framework

The NIC SAFE-ACT sequence

S — Scan for danger. Check fire, electricity, traffic, smoke, chemicals, violence, structural hazards, bodily fluids and other threats. Do not enter an unsafe scene.

A — Assess from a safe position. Check responsiveness, normal breathing, severe bleeding, choking and obvious injury. Do not delay emergency activation when the situation is life-threatening.

F — Find help. Call the local emergency service, facility response, supervisor or clinical professional according to the setting. Send someone for an AED and first-aid equipment where available.

E — Execute trained first aid/BLS. Provide only actions within training, equipment and authority. Follow recognised guidance and device prompts.

A — Assess continuously. Monitor responsiveness, breathing, bleeding and change. Be prepared to update emergency responders.

C — Communicate. Give concise information: location, what happened, current signs, actions taken, hazards and access instructions.

T — Transfer and document. Hand over to responders and complete the approved care record, incident form and follow-up communication.

Safety rule: Do not become the second casualty. Personal safety and emergency activation come before attempting a technique in a dangerous scene.

10. Complete module structure

Module	Title	Lessons	Assessment
1	Safety, Recognition and Emergency Activation	1. First Aid and BLS; 2. Scene Safety and Initial Assessment; 3. Calling for Help and Handover	6 questions
2	BLS, CPR and AED Awareness	4. Recognising Cardiac Arrest; 5. Adult CPR; 6. AED Awareness and Practical Limits	6 questions
3	Choking, Bleeding, Shock and Collapse	7. Choking; 8. Severe Bleeding and Shock; 9. Fainting and Collapse	6 questions
4	Seizures, Burns, Falls, Head Injury and Poisoning	10. Seizures; 11. Burns; 12. Falls and Head Injury; 13. Poisoning and Exposure	7 questions
5	Breathing, Allergic and Diabetic Emergencies	14. Anaphylaxis and Breathing Difficulty; 15. Diabetic Emergencies; 16. Special Caregiver Considerations	7 questions
6	Integrated Emergency Practice, Reporting and Skills Verification	17. Documentation and Incident Reporting; 18.	7 questions

11. Detailed lesson outline

Lesson	Primary purpose
1	Understand first aid, BLS and scope
2	Make scenes safe and perform initial assessment
3	Activate help and communicate location and risk
4	Recognise possible cardiac arrest
5	Explain adult CPR without implying online physical competence
6	Explain AED operation and safety
7	Respond to mild and severe choking
8	Control severe bleeding and recognise shock
9	Respond to fainting and collapse
10	Provide safe seizure first aid
11	Provide basic burn first aid and escalate
12	Respond safely to falls and head injury
13	Respond to poisoning and exposure
14	Recognise anaphylaxis and breathing difficulty
15	Recognise diabetic emergencies and follow the plan
16	Adapt communication and escalation for children, infants, older adults and disability
17	Communicate, document and report emergencies

12. Full lesson content

Module 1 — Safety, Recognition and Emergency Activation

Lesson 1. First Aid and BLS

Why this matters

First aid provides immediate help for illness or injury. BLS focuses on life-threatening emergencies and resuscitation support. Caregivers may need both in one shift.

Learning objectives

The learner should be able to distinguish the terms, explain the caregiver role and identify the online/practical distinction.

Core instructional content

First aid may include protecting a person from further harm, controlling external bleeding, cooling a burn, protecting an injured area, supporting a person after a seizure and activating emergency help. BLS concerns recognition of cardiac arrest or possible arrest, chest compressions, rescue breathing where trained and appropriate, AED use and continuing until help takes over or stopping criteria apply.

Online material can explain sequence and judgement. It cannot prove that the learner can place hands correctly, compress effectively, ventilate safely or operate a specific AED. Practical verification is separate.

Case scenario

A caregiver completes an online CPR lesson and tells an employer, “I am now certified in CPR.”

Best response: The learner may have completed online CPD knowledge, but hands-on CPR competence requires supervised practical verification.

Common mistakes / professional risks

Overclaiming certification, treating a video as practice, delaying help while searching for instructions and acting beyond training.

Key practice points

1. First aid and BLS overlap but are not identical.
2. Safety and emergency activation come first.
3. Online knowledge is not physical competence.
4. Follow current recognised guidance.
5. Use local procedures.

Lesson assessment

1. **MCQ:** BLS focuses particularly on: A) routine hygiene; B) life-threatening emergencies and resuscitation; C) diagnosis; D) medication prescribing. **Answer: B. Objective: 1.**
2. **Scenario:** Online learner claims practical CPR certification. **Answer:** Explain the need for separate skills verification. **Objective: 12.**
3. **True/False:** First aid and BLS mean exactly the same thing. **Answer: False. Objective: 1.**
4. **What should not happen?** Delay emergency activation to watch a video. **Answer:** Call for help and act within training. **Objective: 2–4.**
5. **Application:** Name three first-aid examples. **Answer:** Bleeding control, burn cooling, seizure protection or injury protection. **Objective: 1.**
6. **Scope:** Does online completion prove physical competence? **Answer: No. Objective: 12.**

Lesson 2. Scene Safety and Initial Assessment

Why this matters

Emergency response begins with safety. A caregiver injured by fire, electricity, violence, chemicals or traffic cannot help the client.

Learning objectives

The learner should be able to scan for hazards, assess responsiveness and breathing, identify severe bleeding/choking and activate help.

Core instructional content

Scan before approaching. Consider fire, electricity, smoke, traffic, chemicals, violence, structural instability, bodily fluids and unsafe animals or equipment. Use available personal protective equipment and do not enter a dangerous scene.

From a safe position, assess whether the person responds. Check whether breathing appears normal. Look for life-threatening bleeding, choking and obvious injury. Activate emergency help when needed. Do not spend excessive time on a detailed examination before calling.

Applying this in practice

In a Nigerian home, hazards may include generators, exposed wiring, cooking fuel, traffic near the entrance, crowded rooms or limited access for responders. In a facility, know the emergency call system and AED location.

Case scenario

A client collapses beside an exposed electrical cable.

Best response: Do not touch the client until the electrical hazard is isolated by an appropriate person; call for emergency help and protect others from the scene.

Common mistakes / professional risks

Touching live electricity, entering smoke, ignoring violence, delaying activation and assuming a quiet person is breathing normally.

Key practice points

1. Scan for danger.
2. Protect self and others.
3. Assess responsiveness and normal breathing.
4. Look for severe bleeding and choking.
5. Call early.

Lesson assessment

1. **MCQ:** The first question at an emergency scene is: A) Who is to blame? B) Is it safe to approach? C) What is the diagnosis? D) What medication is needed? **Answer: B.**
Objective: 2.
2. **Scenario:** Exposed electrical cable near casualty. **Answer:** Isolate hazard through appropriate help; do not touch. **Objective: 2.**
3. **True/False:** A silent scene is automatically safe. **Answer: False. Objective: 2.**

4. **What should not happen?** Become a second casualty. **Answer:** Do not enter danger. **Objective: 2.**
5. **Application:** Name four initial checks. **Answer:** Responsiveness, normal breathing, severe bleeding, choking or obvious injury. **Objective: 2.**
6. **Scenario:** Person is unresponsive. **Answer:** Activate emergency help and begin appropriate trained response. **Objective: 3–5.**

Lesson 3. Calling for Help and Handover

Why this matters

Emergency help is only useful if responders can find the person and understand what happened.

Learning objectives

The learner should be able to activate local help, provide location information and give a concise handover.

Core instructional content

State the exact location, access point, hazards, what happened, current responsiveness/breathing, major injury, actions taken and what equipment is needed. Stay available for instructions. Send another person to guide responders where possible.

Because Nigerian infrastructure differs by community, learners must know their employer's emergency service, facility code, supervisor and backup route. Do not assume a national number or service is available everywhere without local confirmation.

Case scenario

A caregiver calls for help and says only, "Come quickly."

Best response: Give location, access details, hazard, condition and actions taken, and remain available.

Common mistakes / professional risks

Vague location, failing to mention hazards, calling the wrong route, leaving the casualty without coverage and not handing over.

Key practice points

1. Call through the correct local pathway.

2. Give location and access.
3. Describe facts, not diagnosis.
4. State actions taken.
5. Stay available.

Lesson assessment

1. **MCQ:** Emergency communication should include: A) gossip; B) location, condition, hazards and actions; C) a diagnosis guess; D) blame. **Answer: B. Objective: 3.**
2. **Scenario:** Responder cannot find the house. **Answer:** Give access point and send a guide where safe. **Objective: 3.**
3. **True/False:** Every Nigerian community has identical emergency infrastructure. **Answer: False. Objective: 3.**
4. **What should not happen?** Leave out a chemical hazard. **Answer:** Tell responders about hazards. **Objective: 2–3.**
5. **Application:** Name five handover elements. **Answer:** Location, event, responsiveness/breathing, hazards, actions and equipment needed. **Objective: 3.**
6. **Scenario:** Supervisor does not answer. **Answer:** Use the next authorised/emergency route without delay. **Objective: 3.**

Module 2 — BLS, CPR and AED Awareness

Lesson 4. Recognising Cardiac Arrest

Why this matters

Rapid recognition and early CPR/AED activation are central to survival in cardiac arrest. A caregiver should not wait for certainty from a clinical diagnosis.

Learning objectives

The learner should be able to identify unresponsiveness and absent or abnormal breathing, activate help and begin the trained response.

Core instructional content

A person who is unresponsive and not breathing normally, including gasping or abnormal breathing, should be treated as a possible cardiac arrest. Call for emergency assistance and an AED. Follow the current adult BLS algorithm and local procedure.

If the person has a valid, known care plan or resuscitation decision, follow authorised policy. Caregivers must not independently interpret legal documents or make treatment decisions.

Case scenario

A client is unresponsive and making occasional abnormal gasps.

Best response: Treat as possible cardiac arrest, activate emergency help/AED and begin trained CPR response.

Common mistakes / professional risks

Waiting for a pulse check that delays action, dismissing gasps as normal breathing, searching for a diagnosis and failing to call.

Key practice points

1. Unresponsive plus abnormal/absent normal breathing is an emergency.
2. Activate help and AED.
3. Follow current BLS guidance.
4. Do not diagnose.
5. Respect authorised care plans through proper pathways.

Lesson assessment

1. **MCQ:** Unresponsive with abnormal gasping should be treated as: A) normal sleep; B) possible cardiac arrest; C) routine faint; D) behavioural issue. **Answer: B. Objective: 4.**
2. **Scenario:** Occasional gasps are observed. **Answer:** Activate emergency response and begin trained CPR pathway. **Objective: 4–5.**
3. **True/False:** Abnormal gasping is normal breathing. **Answer: False. Objective: 4.**
4. **What should not happen?** Delay while seeking diagnostic certainty. **Answer:** Act promptly. **Objective: 4.**
5. **Application:** Name three immediate actions. **Answer:** Call for help, send for AED, begin trained CPR. **Objective: 4–6.**
6. **Scope:** Can caregiver independently interpret resuscitation orders? **Answer:** No; follow authorised policy and seek professional guidance. **Objective: 4.**

Lesson 5. Adult CPR

Why this matters

CPR is a hands-on skill. Online explanation cannot establish physical competence.

Learning objectives

The learner should be able to explain the adult CPR sequence and identify the need for practical verification.

Core instructional content

Follow the current recognised adult BLS algorithm. In general, when an adult is unresponsive and not breathing normally, activate emergency assistance, begin chest compressions and use an AED as soon as available. Current AHA guidance uses a compression rate of **100–120 per minute**, adult compression depth of **at least 5 cm and no more than 6 cm**, full recoil and minimising interruptions ¹. Trained rescuers should provide ventilations according to current guidance; standard adult CPR commonly uses cycles of 30 compressions and 2 breaths when breaths are appropriate and the rescuer is trained. Hands-only CPR may be appropriate for some untrained adult bystanders according to dispatcher/instructor guidance.

Continue until trained responders take over, the person shows signs of life, the scene becomes unsafe, the rescuer is exhausted or an authorised stopping instruction applies. Switch rescuers when possible to maintain quality and minimise interruptions.

These technical parameters must be taught with manikin practice and verified against the current adopted guideline before publication. They do not substitute for local instructor training.

Case scenario

An online learner can recite “30:2” but has never practised compressions.

Best response: Knowledge is not demonstrated physical competence; arrange supervised practical verification.

Common mistakes / professional risks

Incorrect hand position, shallow or excessively deep compressions, long pauses, poor recoil, ventilating without training and claiming practical competence.

Key practice points

1. Call and start quickly.
2. Use current adult BLS algorithm.

3. Minimise interruptions.
4. Use an AED as soon as available.
5. Practise and verify hands-on skill.

Lesson assessment

1. **MCQ:** Current adult compression rate is: A) 40–60/min; B) 100–120/min; C) 160–200/min; D) any rate. **Answer: B. Objective: 5.**
2. **MCQ:** Adult compression depth is approximately: A) at least 5 cm but no more than 6 cm; B) 1 cm; C) 10 cm; D) no depth guidance. **Answer: A. Objective: 5.**
3. **Scenario:** Learner memorised CPR but has not practised. **Answer:** Arrange hands-on verification. **Objective: 12.**
4. **True/False:** Long unnecessary pauses are acceptable. **Answer: False. Objective: 5.**
5. **What should not happen?** Claim practical CPR competence from viewing online material. **Answer:** Keep knowledge and skills credentials separate. **Objective: 12.**
6. **Application:** Name four quality principles. **Answer:** Rate, depth, recoil, minimal interruptions, switching rescuers and AED use. **Objective: 5–6.**

Lesson 6. AED Awareness and Practical Limits

Why this matters

An AED can analyse heart rhythm and provide a shock when indicated. Early use may support resuscitation, but the device must be used safely.

Learning objectives

The learner should be able to explain AED purpose, follow prompts and maintain safety.

Core instructional content

Send for an AED while CPR begins. Turn it on, attach pads as shown, follow voice/visual prompts and ensure no one touches the person during analysis or shock. Resume CPR when directed. Do not override prompts or touch the person during analysis.

AED models differ. Workplace-specific training and manufacturer instructions apply. Practical verification should use a trainer device, not a live AED.

Case scenario

The AED announces analysis while a rescuer's hand remains on the person.

Best response: Ensure everyone is clear and nobody touches the person during analysis.

Common mistakes / professional risks

Touching during analysis/shock, ignoring prompts, using a live device for practice, poor pad placement, delaying CPR and failing to check safety.

Key practice points

1. Send for AED early.
2. Follow the device.
3. Ensure everyone is clear.
4. Resume CPR when directed.
5. Practise on an AED trainer.

Lesson assessment

1. **MCQ:** During AED analysis, rescuers should: A) touch the person; B) ensure nobody touches the person; C) remove pads; D) stop all help. **Answer: B. Objective: 6.**
2. **Scenario:** AED model differs from training. **Answer:** Follow its prompts and workplace-specific guidance. **Objective: 6.**
3. **True/False:** Caregivers should override AED instructions. **Answer: False. Objective: 6.**
4. **What should not happen?** Practise on a live AED. **Answer:** Use a trainer device. **Objective: 12.**
5. **Application:** Name four AED steps. **Answer:** Turn on, attach pads, clear during analysis/shock, follow prompts and resume CPR. **Objective: 6.**
6. **Scenario:** Someone reaches toward casualty during shock. **Answer:** Stop and ensure everyone is clear. **Objective: 6.**

Module 3 — Choking, Bleeding, Shock and Collapse

Lesson 7. Choking

Why this matters

Airway obstruction can progress rapidly. The response depends on whether the person can cough, speak and breathe effectively.

Learning objectives

The learner should be able to distinguish mild from severe obstruction and explain adult and unconscious-casualty response.

Core instructional content

If the person can cough and breathe, encourage coughing and monitor. Do not slap or thrust unnecessarily. If the person cannot speak, cough or breathe normally and shows severe obstruction, activate help and provide current trained first aid. Current recognised adult first-aid algorithms commonly use repeated cycles of up to five back blows followed by up to five abdominal thrusts, continuing until the obstruction clears or the person becomes unresponsive; local instructor guidance and current adopted guidance must be followed 3.

If the person becomes unresponsive, lower safely, activate emergency response and begin CPR according to current guidance. Look for a visible object before breaths; do not perform blind finger sweeps. For infants, children, pregnant people or people with significant abdominal size, use the specific current protocol taught by an instructor.

Case scenario

A client cannot speak or cough and is clutching the throat.

Best response: Recognise severe choking, call for help and use the current trained sequence while preparing for deterioration.

Common mistakes / professional risks

Giving food or drink, blind finger sweep, delaying help, using adult technique for infants and failing to monitor after resolution.

Key practice points

1. Distinguish effective cough from severe obstruction.
2. Activate help.
3. Use current trained sequence.
4. Do not blind-sweep.
5. Use special protocols for children/infants/pregnancy.

Lesson assessment

1. **MCQ:** A person who can cough effectively should generally be: A) encouraged to cough and monitored; B) given water; C) forced to lie down; D) given abdominal thrusts immediately. **Answer: A. Objective: 7.**

2. **Scenario:** Person cannot speak, cough or breathe normally. **Answer:** Severe obstruction; activate help and use trained sequence. **Objective: 7.**
3. **True/False:** Blind finger sweeps are recommended. **Answer: False. Objective: 7.**
4. **What should not happen?** Give food or drink during choking. **Answer:** Use current first-aid response. **Objective: 7.**
5. **Application:** Name two special groups requiring protocol adaptation. **Answer:** Infants, children, pregnant people or people with significant abdominal size. **Objective: 7.**
6. **Scenario:** Person becomes unresponsive. **Answer:** Activate emergency response and begin CPR pathway. **Objective: 7.**

Lesson 8. Severe Bleeding and Shock

Why this matters

Life-threatening bleeding needs immediate action. Shock can develop when the body is not receiving enough effective circulation.

Learning objectives

The learner should be able to recognise severe bleeding, apply trained direct pressure, seek urgent help and identify shock signs.

Core instructional content

Ensure scene safety and use gloves or a barrier where available. Apply firm direct pressure with appropriate dressing where suitable. Add dressings rather than removing a soaked one. Activate emergency assistance. Monitor the person and reassure.

Tourniquets are not routine for every bleed. They should be used only in circumstances covered by current recognised guidance and practical training, with an appropriate device and clear handover. Do not improvise invasive procedures.

Possible shock signs include pale or clammy skin, weakness, dizziness, altered responsiveness, rapid breathing, anxiety or restlessness. Keep the person appropriately positioned according to the situation and current guidance. Avoid unnecessary food or drink if emergency care may be required.

Case scenario

A client has heavy bleeding from a limb and becomes pale and confused.

Best response: Apply trained direct pressure, call urgent help, monitor and communicate possible shock; use a tourniquet only if covered by current training and guidance.

Common mistakes / professional risks

Removing soaked dressings, delaying help, giving food/drink, applying improvised tourniquets and diagnosing the cause of shock.

Key practice points

1. Protect yourself.
2. Direct pressure where appropriate.
3. Call urgently.
4. Monitor for shock.
5. Use tourniquets only under current guidance and training.

Lesson assessment

1. **MCQ:** Severe bleeding requires: A) direct pressure where appropriate and urgent help; B) waiting; C) food; D) routine review. **Answer: A. Objective: 8.**
2. **Scenario:** Dressing is soaked. **Answer:** Add dressing and maintain pressure; follow trained guidance. **Objective: 8.**
3. **True/False:** Tourniquets are routine for all bleeding. **Answer: False. Objective: 8.**
4. **What should not happen?** Give food or drink during serious bleeding. **Answer:** Avoid unnecessary intake and seek help. **Objective: 8.**
5. **Application:** Name four possible shock signs. **Answer:** Pale/clammy, weakness, dizziness, altered responsiveness, rapid breathing or restlessness. **Objective: 8.**
6. **Scenario:** No gloves available but immediate danger exists. **Answer:** Protect self as far as possible, use barrier/material if available and activate help. **Objective: 8.**

Lesson 9. Fainting and Collapse

Why this matters

A simple faint can resemble or precede a serious condition. Sudden collapse requires assessment and escalation.

Learning objectives

The learner should be able to protect a person after fainting, reassess responsiveness and breathing and recognise escalation triggers.

Core instructional content

Protect the person from further injury, assess responsiveness and normal breathing and call for help if recovery is not prompt or the cause is unclear. Do not immediately stand a person who has fainted. Follow current guidance and workplace procedure.

Escalate urgently for persistent unresponsiveness, abnormal breathing, chest pain, severe headache, injury, seizure, repeated fainting, pregnancy concern, severe bleeding, worsening condition or other danger signs.

Case scenario

A client faints, wakes briefly and wants to walk immediately.

Best response: Keep the person safe, reassess, avoid immediate unsupported walking and seek appropriate review, especially if the cause is unclear.

Common mistakes / professional risks

Giving food/drink immediately, standing the person abruptly, dismissing repeated collapse and failing to assess injury.

Key practice points

1. Protect from falls.
2. Reassess responsiveness and breathing.
3. Do not stand abruptly.
4. Identify red flags.
5. Escalate uncertainty.

Lesson assessment

1. **MCQ:** After a faint, the caregiver should first: A) make the person walk; B) protect, reassess and seek guidance; C) give medication; D) ignore. **Answer: B. Objective: 9.**
2. **Scenario:** Person remains unresponsive. **Answer:** Activate emergency response and follow BLS pathway. **Objective: 9.**
3. **True/False:** Every faint is harmless. **Answer: False. Objective: 9.**
4. **What should not happen?** Stand the person abruptly. **Answer:** Keep safe and reassess. **Objective: 9.**
5. **Application:** Name four escalation triggers. **Answer:** Persistent unresponsiveness, abnormal breathing, chest pain, injury, seizure, repeated fainting or worsening. **Objective: 9.**

6. **Scenario:** Faint followed by head strike. **Answer:** Treat as possible injury and escalate.
Objective: 9, 12.

Module 4 — Seizures, Burns, Falls, Head Injury and Poisoning

Lesson 10. Seizures

Why this matters

A seizure can cause sudden loss of awareness and movement. The immediate priority is preventing injury and monitoring breathing, not restraining or diagnosing.

Learning objectives

The learner should be able to protect the person, time the seizure, maintain privacy and escalate when indicated.

Core instructional content

Clear nearby hazards, cushion the head if safe, protect privacy and time the event. Do not restrain, place objects in the mouth or force food/drink. Do not move the person unnecessarily during active movement unless there is immediate danger. After the seizure, check breathing and position according to current guidance and training. Stay with the person and reassure when awareness returns.

Seek emergency help for a first known seizure, prolonged or repeated seizure, injury, breathing difficulty, pregnancy, seizure in water, failure to regain consciousness or other concern. Do not diagnose seizure type.

Case scenario

A family member tries to put a spoon in the client's mouth.

Best response: Stop the unsafe action, protect the person from hazards and explain that nothing should be placed in the mouth.

Common mistakes / professional risks

Restraint, objects in mouth, food/drink, crowding, filming, unnecessary movement and failure to time or escalate.

Key practice points

1. Protect from injury.
2. Time the seizure.
3. Maintain privacy.
4. Nothing in the mouth.
5. Monitor and escalate.

Lesson assessment

1. **MCQ:** During a seizure, the caregiver should: A) restrain; B) clear hazards and time it; C) place a spoon in the mouth; D) give water. **Answer: B. Objective: 10.**
2. **Scenario:** Family wants to restrain. **Answer:** Prevent unsafe restraint and protect from hazards. **Objective: 10.**
3. **True/False:** Food or drink should be forced during a seizure. **Answer: False. Objective: 10.**
4. **What should not happen?** Film the person for social media. **Answer:** Protect privacy. **Objective: 10.**
5. **Application:** Name four escalation triggers. **Answer:** First seizure, prolonged/repeated seizure, injury, breathing difficulty, pregnancy, water or failure to recover. **Objective: 10.**
6. **Scenario:** Person is not breathing normally afterward. **Answer:** Activate emergency/BLS response. **Objective: 10.**

Lesson 11. Burns

Why this matters

Burn first aid should remove danger, cool safely according to current guidance and protect the injury while seeking assessment when needed.

Learning objectives

The learner should be able to explain basic burn response, identify electrical/chemical emergencies and avoid harmful home remedies.

Core instructional content

Remove the person from danger if safe. Cool the burn using cool running water according to current first-aid guidance; do not use ice, butter, toothpaste, oils or other harmful home remedies. Remove jewellery or clothing near the burn only if not stuck. Cover/protect as trained and seek emergency assessment for large, deep, facial, airway, electrical, chemical, circumferential or otherwise serious burns.

For chemical exposure, protect yourself, remove the person from contamination where safe and follow emergency/chemical safety guidance. Electrical burns require power isolation and medical assessment.

Case scenario

A caregiver wants to apply toothpaste to a burn.

Best response: Do not use the home remedy; cool and protect according to current guidance and seek appropriate assessment.

Common mistakes / professional risks

Ice, toothpaste, butter, breaking blisters, peeling stuck clothing and entering electrical danger.

Key practice points

1. Remove danger safely.
2. Cool according to current guidance.
3. Avoid harmful remedies.
4. Escalate serious burns.
5. Protect yourself from chemicals/electricity.

Lesson assessment

1. **MCQ:** A harmful burn remedy is: A) toothpaste; B) appropriate cooling; C) protection; D) emergency help. **Answer: A. Objective: 11.**
2. **Scenario:** Electrical burn with power hazard. **Answer:** Do not touch until power is isolated; activate emergency help. **Objective: 11.**
3. **True/False:** Ice is routinely recommended on burns. **Answer: False. Objective: 11.**
4. **What should not happen?** Peel off clothing stuck to the burn. **Answer:** Seek trained help and protect the area. **Objective: 11.**
5. **Application:** Name four serious-burn triggers. **Answer:** Large/deep, face/airway, electrical, chemical or circumferential burn. **Objective: 11.**
6. **Scenario:** Chemical splashes caregiver. **Answer:** Protect self, follow chemical emergency guidance and seek help. **Objective: 11.**

Lesson 12. Falls and Head Injury

Why this matters

Lifting a fallen person automatically can worsen an injury. Caregivers should assess immediate danger and seek appropriate help.

Learning objectives

The learner should be able to respond to falls, suspected fracture, head/neck/back injury and inability to get up.

Core instructional content

Check scene safety and responsiveness. Do not automatically lift the person. Keep them still and comfortable where safe, call for assistance and follow the organisation's falls procedure. Do not move a person with suspected spinal injury unless immediate danger requires it or trained responders direct it.

Escalate for loss of consciousness, confusion, severe pain, suspected fracture, bleeding, repeated vomiting, seizure, worsening condition, anticoagulant concern or any head/neck/back injury. Do not diagnose concussion or internal injury.

Case scenario

A client falls and says they are fine but cannot stand.

Best response: Do not lift automatically; keep safe, assess and seek assistance according to procedure.

Common mistakes / professional risks

Pulling by the arms, standing quickly, ignoring head strike, delaying for paperwork and assuming no pain means no injury.

Key practice points

1. Do not automatically lift.
2. Assess safety and responsiveness.
3. Seek help.
4. Monitor for red flags.
5. Document and report.

Lesson assessment

1. **MCQ:** After a fall, the caregiver should: A) lift immediately; B) assess and seek help; C) leave; D) give medication. **Answer: B. Objective: 12.**

2. **Scenario:** Person cannot get up. **Answer:** Do not lift automatically; call assistance and follow procedure. **Objective: 12.**
3. **True/False:** Confusion after a fall is an escalation trigger. **Answer: True. Objective: 12.**
4. **What should not happen?** Diagnose concussion. **Answer:** Record signs and escalate. **Objective: 12.**
5. **Application:** Name five red flags. **Answer:** Loss of consciousness, confusion, severe pain, fracture, bleeding, vomiting, seizure or worsening. **Objective: 12.**
6. **Scenario:** Head strike followed by vomiting. **Answer:** Urgent escalation. **Objective: 12.**

Lesson 13. Poisoning and Exposure

Why this matters

Medication overdose, chemicals and inhalation exposures can harm both client and caregiver. Unsafe home treatment may worsen the situation.

Learning objectives

The learner should be able to protect self, remove danger safely, preserve information and seek expert help.

Core instructional content

Move away from fumes or contamination where safe. Do not induce vomiting unless specifically instructed by an appropriate authority. Do not give home remedies. Retain packaging, container, product name and estimated time/amount where safe, without delaying emergency care. Contact emergency services or poison information support where available and follow local procedure.

If the person is unresponsive or breathing abnormally, activate BLS/emergency response. Protect responders by sharing the substance and hazard information.

Case scenario

A client may have swallowed cleaning chemical and a relative suggests inducing vomiting.

Best response: Do not induce vomiting; protect, call emergency/poison support and retain the container for responders if safe.

Common mistakes / professional risks

Inducing vomiting, giving milk or other remedies without instruction, handling chemicals without protection and delaying emergency activation.

Key practice points

1. Protect yourself.
2. Move from danger where safe.
3. Do not induce vomiting unless instructed.
4. Retain packaging information.
5. Call appropriate help.

Lesson assessment

1. **MCQ:** In suspected poisoning, caregivers should: A) induce vomiting; B) seek expert help and retain packaging where safe; C) give random food; D) wait. **Answer: B. Objective: 13.**
2. **Scenario:** Chemical fumes in a room. **Answer:** Move away if safe, protect self and call help. **Objective: 13.**
3. **True/False:** Home remedies are automatically safe. **Answer: False. Objective: 13.**
4. **What should not happen?** Handle an unknown chemical bare-handed. **Answer:** Protect self and await guidance. **Objective: 13.**
5. **Application:** Name three useful information items. **Answer:** Product/container, time and estimated amount/exposure. **Objective: 13.**
6. **Scenario:** Client is unresponsive after exposure. **Answer:** Activate emergency/BLS response and communicate hazard. **Objective: 13.**

Module 5 — Breathing, Allergic and Diabetic Emergencies

Lesson 14. Anaphylaxis and Breathing Difficulty

Why this matters

Severe allergic reaction and serious breathing difficulty can deteriorate rapidly.

Learning objectives

The learner should be able to recognise danger signs, activate emergency help, reassure and follow authorised emergency plans.

Core instructional content

Possible anaphylaxis signs include swelling of lips/tongue/throat, wheezing, breathing difficulty, widespread reaction, collapse or rapidly worsening symptoms. Severe breathing

difficulty may include inability to speak normally, cyanosis awareness, abnormal breathing or worsening respiratory distress.

Call urgent help. Position according to current guidance and the person's emergency plan; do not force them to lie flat if that worsens breathing. Reassure and monitor. If an adrenaline/epinephrine auto-injector is prescribed and available, assistance with it should occur only where the caregiver is trained and authorised under applicable procedure. Do not administer prescription medication outside scope or policy.

Case scenario

A client develops facial swelling and difficulty breathing after food.

Best response: Activate emergency response immediately, follow the authorised emergency plan and support trained use of prescribed equipment if permitted.

Common mistakes / professional risks

Delaying help, giving food/drink, making the person walk, administering unapproved medication and assuming a mild rash cannot worsen.

Key practice points

1. Treat severe allergic reaction as urgent.
2. Call for help.
3. Follow emergency plan and training.
4. Position and reassure appropriately.
5. Monitor continuously.

Lesson assessment

1. **MCQ:** Facial swelling plus breathing difficulty suggests: A) routine rash; B) possible anaphylaxis; C) minor discomfort; D) no action. **Answer: B. Objective: 14.**
2. **Scenario:** Prescribed auto-injector is available but caregiver is untrained. **Answer:** Activate help and follow policy; do not improvise administration. **Objective: 14.**
3. **True/False:** Severe breathing difficulty can wait for routine review. **Answer: False. Objective: 14.**
4. **What should not happen?** Force the person to walk. **Answer:** Reassure, position appropriately and call. **Objective: 14.**
5. **Application:** Name four danger signs. **Answer:** Swelling, wheeze, breathing difficulty, widespread reaction, collapse or inability to speak normally. **Objective: 14.**

6. **Scenario:** Client becomes unresponsive. **Answer:** Activate BLS response. **Objective: 14.**

Lesson 15. Diabetic Emergencies

Why this matters

Hypoglycaemia, hyperglycaemia and altered consciousness require recognition and following the person's care plan. Caregivers must not adjust medication independently.

Learning objectives

The learner should be able to recognise possible diabetic emergency, provide only authorised support and escalate altered consciousness.

Core instructional content

Possible low-blood-glucose signs include sweating, shaking, weakness, hunger, confusion, unusual behaviour or altered responsiveness. High blood glucose may involve thirst, frequent urination, weakness, vomiting or worsening illness, but caregivers must not diagnose from signs alone.

If the person is awake and able to swallow, follow the documented emergency plan and authorised instructions. Do not give anything by mouth to a person with altered consciousness, seizure or unsafe swallowing. Activate urgent help for altered consciousness, breathing difficulty, persistent vomiting or deterioration. Do not change insulin or other medication unless explicitly authorised within the caregiver's role and policy.

Case scenario

A confused client with diabetes is drowsy and unable to swallow safely. A relative asks the caregiver to pour juice into the mouth.

Best response: Do not give oral intake to an unsafe-swallowing or altered-consciousness person; activate urgent help and follow the emergency plan.

Common mistakes / professional risks

Forcing food/drink, changing insulin, diagnosing from one sign and delaying help.

Key practice points

1. Recognise possible signs.
2. Check consciousness and swallowing safety.
3. Follow the care plan.

4. Do not adjust medication.
5. Escalate deterioration.

Lesson assessment

1. **MCQ:** An altered-consciousness person should be given juice by mouth: A) always; B) never without safe swallowing and authorised guidance; C) rapidly; D) as punishment. **Answer: B. Objective: 15.**
2. **Scenario:** Drowsy client cannot swallow. **Answer:** Nothing by mouth; activate urgent help. **Objective: 15.**
3. **True/False:** Caregivers may independently change insulin. **Answer: False. Objective: 15.**
4. **What should not happen?** Force food or drink. **Answer:** Protect airway and escalate. **Objective: 15.**
5. **Application:** Name four possible low-glucose signs. **Answer:** Sweating, shaking, weakness, hunger, confusion, behaviour change or altered response. **Objective: 15.**
6. **Scenario:** Persistent vomiting and weakness. **Answer:** Urgent clinical/emergency escalation. **Objective: 15.**
7. **Documentation:** Record signs, plan followed, actions and contacts. **Answer:** Correct. **Objective: 17.**

Lesson 16. Special Caregiver Considerations

Why this matters

Children, infants, older adults and people with disability may require different equipment, communication and guidance. One adult algorithm cannot be applied to every person.

Learning objectives

The learner should be able to identify when age, size, disability, pregnancy, frailty or communication needs require specialised guidance.

Core instructional content

Use the current child and infant BLS/first-aid protocol taught by an appropriately qualified instructor. Do not improvise adult depth or technique. Consider smaller equipment, communication supports and the need for a parent/guardian or clinical professional according to policy.

Older adults may have frailty, anticoagulant use, hearing impairment, cognitive impairment or atypical signs. Disability may affect communication, positioning or access. Pregnancy, obesity, spinal injury, tracheostomy, implanted devices and other conditions may require specialised procedures.

The caregiver's role is to recognise complexity, keep safe, call for help and follow current trained guidance—not to invent a modified protocol.

Case scenario

A caregiver trained only in adult CPR faces an infant collapse.

Best response: Activate emergency help, follow dispatcher/instructor guidance and use the current infant protocol only if trained.

Common mistakes / professional risks

Using adult technique on infants, ignoring communication needs, moving a frail person roughly and assuming no visible injury means no risk.

Key practice points

1. Age and condition affect response.
2. Use current specialised protocol.
3. Communicate accessibly.
4. Protect frail and disabled people from additional injury.
5. Escalate uncertainty.

Lesson assessment

1. **MCQ:** Adult CPR technique should be used for infants: A) always; B) only if current guidance specifically says so; C) as a guess; D) without training. **Answer: B. Objective: 16.**
2. **Scenario:** Frail older person falls. **Answer:** Use careful assessment, do not lift automatically and escalate risk. **Objective: 16.**
3. **True/False:** Disability may affect communication and positioning. **Answer: True. Objective: 16.**
4. **What should not happen?** Improvise a child protocol. **Answer:** Use current trained guidance. **Objective: 16.**
5. **Application:** Name four groups requiring additional consideration. **Answer:** Infants, children, older adults, pregnancy, disability, frailty or special devices. **Objective: 16.**

6. **Scenario:** Person cannot hear verbal instructions. **Answer:** Use accessible communication and call help. **Objective: 16.**
7. **Scope:** What should caregiver do if unsure? **Answer:** Activate help and seek current authorised guidance. **Objective: 16.**

Module 6 — Integrated Emergency Practice, Reporting and Skills Verification

Lesson 17. Documentation and Incident Reporting

Why this matters

Emergency response must be handed over and recorded. Documentation supports continuity, accountability, incident learning and follow-up, but it never replaces urgent escalation.

Learning objectives

The learner should be able to record event, signs, actions, contacts, response and follow-up, and complete incident/near-miss reports.

Core instructional content

Record date/time, location, what happened, scene hazards, responsiveness/breathing, visible injury, exact statements where relevant, first-aid/BLS actions, equipment used, response, emergency contacts, advice received, handover and follow-up. Use objective language. Do not diagnose, blame or claim a technique worked if it was not assessed.

Complete incident reports for falls, injuries, medication errors, near misses, equipment failures, choking, suspected abuse and other events according to policy. Documenting is not the same as escalating.

Case scenario

A caregiver writes “CPR successful” without knowing the outcome.

Best response: Record observable actions and handover facts, not an unsupported outcome claim.

Common mistakes / professional risks

Delayed documentation, invented timings, blame, missing contacts, public disclosure and treating the report as optional.

Key practice points

1. Escalate first when urgent.
2. Record facts and times.
3. Include actions and response.
4. Complete incident/near-miss reports.
5. Protect confidentiality.

Lesson assessment

1. **MCQ:** Emergency documentation should include: A) diagnosis guess; B) event, signs, actions, contacts and follow-up; C) gossip; D) blame. **Answer: B. Objective: 17.**
2. **Scenario:** Caregiver writes “CPR successful.” **Answer:** Replace with observable facts and known outcome. **Objective: 17.**
3. **True/False:** Documentation replaces emergency activation. **Answer: False. Objective: 17.**
4. **What should not happen?** Invent a time. **Answer:** Record accurately and identify uncertainty. **Objective: 17.**
5. **Application:** Name six record fields. **Answer:** Time, location, event, signs, actions, equipment, contacts, response and follow-up. **Objective: 17.**
6. **Scenario:** Wrong AED pads discovered before use. **Answer:** Protect safety and report as near miss according to policy. **Objective: 17.**

Lesson 18. Integrated Judgement and Practical Pathways

Why this matters

Emergency care combines safety, recognition, activation, first aid/BLS, monitoring, communication and documentation. A credential must accurately represent what was demonstrated.

Learning objectives

The learner should be able to apply SAFE-ACT to a complex emergency and distinguish online knowledge from practical endorsement.

Core instructional content

A practical decision sequence is: Is the scene safe? Is the person responsive and breathing normally? Is there severe bleeding, choking or obvious injury? Who is calling for help? What

trained action is appropriate? Is an AED needed? What must be monitored? What must be handed over and documented?

The online pathway ends with knowledge assessment. The practical pathway adds instructor-supervised demonstration of required skills, remediation and reassessment. NIC should not issue a practical endorsement without identity verification, equipment, assessor qualification, observed performance and recorded result.

Case scenario

A client collapses near a generator, another client is choking and the facility has one AED.

Best response: Ensure scene safety, allocate helpers, activate emergency response, prioritise immediate life threat, retrieve AED/first-aid equipment, provide trained response and hand over/document both events.

Common mistakes / professional risks

Entering danger, abandoning one casualty without help, claiming competence, delaying emergency call, ignoring the second emergency and failing to document.

Key practice points

1. Safety first.
2. Prioritise life threat.
3. Delegate and call.
4. Act within training.
5. Separate knowledge and practical credentials.

Lesson assessment

1. **Integrated scenario:** Collapse near generator. **Answer:** Isolate danger, call help and do not touch until safe. **Objective: 2–4.**
2. **MCQ:** Practical endorsement requires: A) video viewing; B) supervised skills verification; C) a self-declaration; D) attendance only. **Answer: B. Objective: 12.**
3. **True/False:** Online completion alone proves AED competence. **Answer: False. Objective: 12.**
4. **What should not happen?** Prioritise paperwork over emergency safety. **Answer:** Act first, document afterward. **Objective: 17–18.**
5. **Application:** List SAFE-ACT. **Answer:** Scan, Assess, Find help, Execute trained action, Assess continuously, Communicate, Transfer/document. **Objective: 18.**

6. **Scenario:** Two emergencies occur simultaneously. **Answer:** Call/allocate helpers, prioritise life threat and coordinate responders. **Objective: 18.**
7. **Documentation:** Record both events and handover. **Answer:** Correct. **Objective: 17–18.**

13. Lesson assessments

Each lesson contains six or seven assessment items with immediate feedback. Technical CPR, choking, AED and first-aid questions must be reviewed by an appropriately qualified clinical/resuscitation educator before publication. Item banks should be updated when adopted guidance changes.

14. Module assessments

Module 1 assessment

1. **Scenario:** Unsafe electrical scene. **Answer:** Do not approach until safe; call help. **LO:** 2–3.
2. **MCQ:** First aid and BLS are related but distinct. **Answer:** Correct. **LO:** 1.
3. **Scenario:** Vague emergency call. **Answer:** Give location, hazard, condition and actions. **LO:** 3.
4. **What should not happen?** Become second casualty. **LO:** 2.
5. **True/False:** Online viewing proves physical competence. **Answer:** False. **LO:** 12.
6. **Application:** Name initial checks. **Answer:** Responsiveness, breathing, bleeding, choking and injury. **LO:** 2.

Module 2 assessment

1. **Scenario:** Abnormal gasping. **Answer:** Treat as possible cardiac arrest and activate BLS. **LO:** 4–5.
2. **MCQ:** Adult compression rate is 100–120/min. **Answer:** Correct under current cited guidance. **LO:** 5.
3. **Scenario:** Touch during AED analysis. **Answer:** Clear the person. **LO:** 6.
4. **What should not happen?** Claim hands-on skill from online learning. **LO:** 12.
5. **True/False:** AED prompts should be overridden. **Answer:** False. **LO:** 6.
6. **Application:** Name CPR quality principles. **Answer:** Rate, depth, recoil and minimal interruption. **LO:** 5.

Module 3 assessment

1. **Scenario:** Severe choking. **Answer:** Activate help and use current trained response. **LO:** 7.
2. **MCQ:** Effective cough should generally be encouraged and monitored. **Answer:** Correct. **LO:** 7.
3. **Scenario:** Heavy bleeding and pallor. **Answer:** Direct pressure, urgent help and shock monitoring. **LO:** 8.
4. **What should not happen?** Give food during serious bleeding. **LO:** 8.
5. **True/False:** Every faint is harmless. **Answer:** False. **LO:** 9.
6. **Application:** Name collapse red flags. **Answer:** Persistent unresponsiveness, abnormal breathing, injury, chest pain, seizure or worsening. **LO:** 9.

Module 4 assessment

1. **Scenario:** Seizure. **Answer:** Clear hazards, time, protect privacy and do not restrain. **LO:** 10.
2. **MCQ:** Toothpaste is not appropriate burn treatment. **Answer:** Correct. **LO:** 11.
3. **Scenario:** Fall with inability to stand. **Answer:** Do not lift automatically; call help. **LO:** 12.
4. **What should not happen?** Induce vomiting after poisoning. **LO:** 13.
5. **True/False:** Head injury with vomiting warrants escalation. **Answer:** True. **LO:** 12.
6. **Application:** Name serious burn or head-injury triggers. **Answer:** Electrical/chemical/airway burn, loss of consciousness, confusion, vomiting, seizure or worsening. **LO:** 11–12.
7. **Scenario:** Unknown chemical exposure. **Answer:** Protect self, move from danger if safe and call expert help. **LO:** 13.

Module 5 assessment

1. **Scenario:** Facial swelling and breathing difficulty. **Answer:** Urgent anaphylaxis response. **LO:** 14.
2. **MCQ:** Untrained caregiver should not improvise auto-injector administration outside policy. **Answer:** Correct. **LO:** 14.
3. **Scenario:** Altered diabetic client cannot swallow. **Answer:** Nothing by mouth; urgent help. **LO:** 15.
4. **What should not happen?** Adjust insulin independently. **LO:** 15.
5. **True/False:** Infants require age-specific guidance. **Answer:** True. **LO:** 16.

6. **Application:** Name special considerations. **Answer:** Age, frailty, disability, pregnancy, communication and special devices. **LO:** 16.
7. **Scenario:** Severe breathing difficulty. **Answer:** Call, position appropriately, reassure and monitor. **LO:** 14.

Module 6 assessment

1. **Scenario:** “CPR successful” written without outcome. **Answer:** Record observable facts only. **LO:** 17.
2. **MCQ:** Urgent escalation precedes routine documentation. **Answer:** Correct. **LO:** 17.
3. **Scenario:** Wrong equipment discovered before use. **Answer:** Report near miss. **LO:** 17.
4. **What should not happen?** Claim practical endorsement after online course. **LO:** 18.
5. **True/False:** A practical assessment needs an appropriately qualified assessor. **Answer:** True. **LO:** 18.
6. **Application:** List SAFE-ACT. **Answer:** Scan, Assess, Find help, Execute, Assess, Communicate, Transfer/document. **LO:** 18.
7. **Integrated scenario:** Multiple emergencies. **Answer:** Safety, delegation, emergency activation, prioritisation, trained action, handover and documentation. **LO:** 1–12.

15. Final assessment blueprint

The final assessment contains **35 questions** with a recommended pass mark of **80% (28/35)**. **28 questions are scenario/application based.**

Content area	Questions	Number	Scenario/application
Safety, recognition and activation	1–5	5	4
CPR, BLS and AED	6–10	5	4
Choking, bleeding, shock and collapse	11–15	5	4
Seizures, burns, falls and poisoning	16–20	5	4
Anaphylaxis, breathing and diabetes	21–25	5	4
Special populations and documentation	26–30	5	4

Integrated judgement and credential limits	31–35	5	4
Total		35	28

16. Final assessment questions

1. **MCQ:** The first priority at an emergency scene is: A) blame; B) scene safety; C) documentation; D) diagnosis. **Answer: B. LO: 2.**
2. **Scenario:** Exposed electricity near casualty. **Answer:** Do not touch; isolate hazard through appropriate help and call emergency services. **LO: 2.**
3. **True/False:** A caregiver should avoid becoming a second casualty. **Answer: True. LO: 2.**
4. **Scenario:** Emergency caller gives no location. **Answer:** Provide exact location, access, hazard, condition and actions. **LO: 3.**
5. **What should not happen?** Delay help to search for a diagnosis. **Answer:** Recognise, call and act within scope. **LO: 3–4.**
6. **MCQ:** Possible cardiac arrest includes unresponsiveness with: A) normal sleep; B) absent or abnormal normal breathing; C) hunger; D) mild rash. **Answer: B. LO: 4.**
7. **Scenario:** Abnormal gasping. **Answer:** Activate BLS/emergency response and begin trained CPR pathway. **LO: 4–5.**
8. **MCQ:** Current adult compression rate is: A) 40–60/min; B) 100–120/min; C) 200/min; D) none. **Answer: B. LO: 5.**
9. **Scenario:** AED analysing while someone touches casualty. **Answer:** Ensure everyone is clear. **LO: 6.**
10. **True/False:** Online CPR learning proves hands-on skill. **Answer: False. LO: 5–6, 12.**
11. **Scenario:** Person cannot speak, cough or breathe normally. **Answer:** Treat as severe choking, activate help and use current trained response. **LO: 7.**
12. **MCQ:** Effective cough is generally supported by: A) encouraging cough and monitoring; B) giving water; C) blind sweep; D) immediate medication. **Answer: A. LO: 7.**
13. **Scenario:** Heavy bleeding. **Answer:** Direct pressure where appropriate, urgent help and monitoring. **LO: 8.**
14. **What should not happen?** Apply routine improvised tourniquet. **Answer:** Use only current trained guidance. **LO: 8.**
15. **Scenario:** Faint followed by persistent confusion. **Answer:** Urgent escalation. **LO: 9.**

16. **Scenario:** Seizure begins. **Answer:** Clear hazards, time, protect privacy and do not restrain or place objects in mouth. **LO:** 10.
17. **MCQ:** A harmful burn remedy is: A) toothpaste; B) appropriate cooling; C) protection; D) emergency help. **Answer: A. LO:** 11.
18. **Scenario:** Fall and inability to stand. **Answer:** Do not lift automatically; call assistance. **LO:** 12.
19. **True/False:** Vomiting after head injury is an escalation concern. **Answer: True. LO:** 12.
20. **Scenario:** Suspected chemical ingestion. **Answer:** Do not induce vomiting; call expert help and retain packaging if safe. **LO:** 13.
21. **MCQ:** Facial swelling and breathing difficulty may indicate: A) anaphylaxis; B) routine tiredness; C) no emergency; D) minor thirst. **Answer: A. LO:** 14.
22. **Scenario:** Severe breathing difficulty. **Answer:** Call urgent help, position appropriately, reassure and monitor. **LO:** 14.
23. **Scenario:** Altered diabetic client cannot swallow. **Answer:** Nothing by mouth; activate urgent help and follow plan. **LO:** 15.
24. **What should not happen?** Change insulin independently. **Answer:** Escalate and follow authorised plan. **LO:** 15.
25. **True/False:** Infants may require a different protocol from adults. **Answer: True. LO:** 16.
26. **Scenario:** Frail older adult falls and takes anticoagulants. **Answer:** Do not lift automatically; escalate for assessment and report relevant information. **LO:** 16.
27. **MCQ:** Emergency records should include: A) diagnosis guess; B) facts, actions, contacts and follow-up; C) gossip; D) blame. **Answer: B. LO:** 17.
28. **Scenario:** System is offline during emergency. **Answer:** Use approved downtime process after urgent action and preserve sequence. **LO:** 17.
29. **True/False:** Incident reporting is unnecessary when no harm occurred. **Answer: False. LO:** 17.
30. **Scenario:** Wrong AED pads caught before use. **Answer:** Report as a near miss according to policy. **LO:** 17.
31. **Scenario:** Collapse near generator and second person choking. **Answer:** Secure scene, call/allocate helpers, prioritise immediate life threat and coordinate response. **LO:** 2–7, 18.
32. **MCQ:** Practical endorsement requires: A) online viewing; B) supervised skills verification; C) self-declaration; D) attendance only. **Answer: B. LO:** 12.

33. **Scenario:** Learner has memorised CPR but cannot perform compressions. **Answer:** Knowledge completion only; arrange practical verification. **LO:** 12.
34. **What should not happen?** Claim NIC online CPD is statutory workplace certification. **Answer:** Use precise online credential wording. **LO:** 12.
35. **Integrated scenario:** Client collapses in an unsafe room, AED is available and family is filming. **Answer:** Make scene safe, activate help, stop unsafe filming where possible, begin trained BLS/AED response, hand over and document. **LO:** 2–12.

17. Answer key and explanations

Q	Answer	Reasoning
1	B	Safety prevents a second casualty.
2	Do not touch	Electrical hazards must be isolated safely.
3	True	Rescuer safety is essential.
4	Give details	Responders need location, access, hazards and condition.
5	Do not delay	Early activation matters more than diagnosis.
6	B	Abnormal breathing with unresponsiveness is an emergency.
7	Activate BLS	Gasping is not normal breathing.
8	B	100–120/min is the current adult guidance cited.
9	Clear	Touch can compromise analysis/shock safety.
10	False	Physical competence requires supervised practice.
11	Severe choking response	Inability to speak/cough/breathe normally is urgent.

12	A	Effective cough may clear obstruction; monitor.
13	Direct pressure and help	Severe bleeding can rapidly threaten life.
14	Current guidance only	Tourniquet use requires indication, equipment and training.
15	Urgent escalation	Persistent confusion may signal serious cause.
16	Protect and time	Restraint and objects in mouth cause harm.
17	A	Toothpaste and similar remedies can worsen injury.
18	Do not lift	Injury may be worsened by movement.
19	True	Vomiting after head injury is a red flag.
20	No induced vomiting	Expert guidance is required.
21	A	Swelling and breathing difficulty may be anaphylaxis.
22	Call and monitor	Breathing difficulty can deteriorate rapidly.
23	Nothing by mouth	Unsafe swallowing creates aspiration risk.
24	Do not change insulin	Medication adjustment is outside ordinary caregiver scope.
25	True	Infant and child protocols differ.
26	Escalate	Frailty and anticoagulants may increase risk.
27	B	Objective facts support handover and follow-up.
28	Downtime process	Urgent action comes first; record transparently later.

29	False	Near misses support safety learning.
30	Report near miss	No harm does not mean no system risk.
31	Coordinate	Scene safety, delegation and prioritisation are necessary.
32	B	Physical competence must be observed.
33	Knowledge only	Recitation is not performance.
34	Precise wording	NIC cannot imply statutory recognition without verification.
35	Integrated SAFE-ACT	Safety, BLS, privacy, handover and documentation all apply.

18. Practical competency verification framework

Practical verification should be delivered in person or through an approved live practical process by an appropriately qualified instructor/assessor. It should include identity verification, briefing, equipment check, demonstration, feedback, remediation and reassessment.

Skill station	Minimum observed capability
Scene safety and initial assessment	Scans hazards, checks responsiveness/breathing and calls help
Adult CPR	Performs compressions with correct position, rate, depth, recoil and minimal interruption on a manikin
Rescue breathing	Demonstrates only if trained, equipped and assessed under current protocol
AED	Turns on trainer, applies pads, clears during analysis/shock and follows prompts
Choking	Demonstrates current adult response and identifies child/infant adaptations

Severe bleeding	Uses gloves/barrier, direct pressure, dressing and escalation; tourniquet only if separately trained
Seizure	Clears hazards, times, protects and avoids unsafe actions
Burns/falls	Uses safe first aid and avoids harmful movement/remedies
Emergency communication	Gives location, condition, hazards and actions
Documentation	Completes objective record and incident report

A practical endorsement should expire or require reassessment according to the instructor's recognised certification system, employer policy and current guidance. NIC should not create a renewal period that conflicts with the practical provider's requirements.

19. Practical competency checklist

Adult CPR and AED checklist

Criterion	Pass / Remediate	Assessor notes
Checks scene safety		
Checks responsiveness and breathing		
Activates emergency help		
Requests AED/equipment		
Positions hands correctly		
Maintains current compression rate		
Maintains current depth and recoil		
Minimises interruptions		
Provides breaths only if trained and assessed		

Uses AED trainer safely		
Clears during analysis/shock		
Resumes CPR as directed		
Gives concise handover		
Documents event objectively		

Critical-fail behaviours

Critical failure includes entering danger, failing to call emergency help, unsafe AED contact during analysis/shock, claiming skills not demonstrated, blind finger sweep, placing objects in a seizure casualty's mouth, force-feeding an altered-consciousness person, improvised invasive treatment or failing to disclose an emergency during handover.

20. Online CPD completion requirements

Pathway A — Online CPD: Complete all six modules and required learning activities, attempt each module assessment, achieve at least **80% (28/35)** in the final assessment and acknowledge the scope statement. The resulting certificate must state **online CPD completion** and must not imply hands-on competence, statutory certification or licensure.

NIC may allow up to three final-assessment attempts, with remediation after failure. Retain learner identity, scores, attempts, completion date, curriculum version and scope acknowledgement.

21. CPD + practical verification requirements

Pathway B — CPD plus practical verification: Meet all online requirements and successfully demonstrate the specified practical skills to an appropriately qualified instructor/assessor using suitable equipment and a documented checklist.

The practical endorsement should name the skills actually assessed, the assessment date, assessor identity/qualification, equipment used, result, expiry/reassessment date and any limitations. It must not claim statutory workplace recognition unless independently verified.

22. Certificate wording

A. Online CPD completion certificate

Certificate title: NIC CPD Micro-Credential: First Aid & Basic Life Support for Professional Caregivers

This certificate recognises successful completion of NIC online continuing professional development in caregiver-level emergency recognition, scene safety, emergency activation, first-aid awareness, BLS/CPR and AED awareness, choking, bleeding, shock, collapse, seizures, burns, falls, poisoning, allergic and diabetic emergencies, special considerations, emergency communication, documentation and incident reporting. **This certificate represents online CPD knowledge completion only. It does not prove hands-on competence in CPR, rescue breathing, AED use, choking response or first-aid procedures. It is not a regulated clinical qualification, nursing qualification, professional licence, statutory workplace certification or legal authorisation.**

B. CPD with verified practical competency certificate/endorsement

This certificate recognises completion of the NIC online First Aid & Basic Life Support CPD and successful practical verification of the specific skills listed in the attached assessor record on [date]. The practical verification was conducted by [assessor/provider] using [equipment]. **This endorsement represents demonstrated competence only in the specified assessed skills at the time of assessment. It is not a regulated clinical qualification, nursing qualification, professional licence or automatic statutory workplace certification. The holder must follow current guidance, employer policy and local emergency procedures.**

23. Digital badge specifications

A. NIC First Aid & BLS — Knowledge Badge

Field	Specification
Description	Recognises online caregiver CPD in emergency recognition, safety, first-aid/BLS awareness, activation, communication and documentation.
Competencies	SAFE-ACT, emergency recognition, CPR/AED knowledge, choking, bleeding, common emergencies and reporting
CPD hours	5–6 online hours
Evidence	Six modules, module assessments, final score of at least 80%, scope acknowledgement

Verification	NIC badge URL/QR, certificate ID, date and curriculum version
Limitation	Does not represent hands-on competence, licence or statutory certification
Review	Recommend review every 24 months or sooner when guidance changes

B. NIC First Aid & BLS — Practical Competency Verified Badge

Field	Specification
Description	Recognises online CPD plus observed practical verification of specified first-aid/BLS skills.
Competencies	Only those listed on the assessor record, such as adult CPR, AED trainer, choking, bleeding and emergency communication
CPD hours	5–6 online hours plus separately recorded practical assessment
Evidence	Online completion, assessor identity/qualification, checklist, date, equipment and result
Verification	NIC badge URL/QR, certificate ID, assessor record and expiry/reassessment date
Limitation	Not licensure or automatic statutory workplace certification
Renewal	Follow practical provider/employer/current-guidance requirements; recommend review at least every 24 months unless a recognised provider requires sooner

24. NIC portal course description

First Aid & Basic Life Support for Professional Caregivers is a 5–6 hour NIC online CPD micro-credential for caregivers who may be first present when a client becomes suddenly unwell or injured.

Learners practise recognising danger, ensuring safety, checking responsiveness and breathing, activating emergency assistance, explaining adult CPR and AED response, responding to choking, severe bleeding, shock, collapse, seizures, burns, falls, poisoning, anaphylaxis, breathing difficulty and diabetic emergencies, communicating with responders and documenting events.

The portal must display two distinct pathways: **Online CPD Knowledge Completion** and **CPD Plus Practical Skills Verification**. Viewing online content does not prove hands-on CPR, AED, choking or first-aid competence.

25. Downloadable job aids

A. Emergency Response Quick Reference

Scan danger; assess responsiveness/breathing; look for severe bleeding/choking; call the local emergency pathway; send for AED/equipment; act within training; monitor; hand over; document.

B. CPR/BLS Action Card

For adult unresponsive and not breathing normally: ensure safety, call for help/AED, begin current trained CPR, follow current rate/depth/recoil guidance, minimise interruption, use AED prompts and continue until appropriate handover or stopping criteria.

C. Choking Response Guide

Effective cough: encourage and monitor. Severe obstruction: activate help and use current trained adult sequence. Unresponsive: CPR/emergency pathway. Never blind-sweep or place objects in the mouth. Use special child/infant/pregnancy guidance.

D. Severe Bleeding First-Aid Guide

Protect self; use barrier; apply direct pressure; call urgent help; monitor shock; use tourniquet only when current guidance and practical training apply; hand over and document.

E. Emergency Handover Template

Location; what happened; hazards; responsiveness/breathing; injury/bleeding/choking; actions; equipment; response; contacts; instructions; follow-up.

F. First-Aid Incident Documentation Form

Date/time; location; event; observations; exact statements; action; equipment; response; emergency contact; handover; incident/near-miss number; follow-up.

G. Emergency Equipment Checklist

First-aid kit; gloves/barriers; dressings; burns materials; communication device; AED location/status; trainer availability; emergency numbers; access route; backup power/communication.

H. Practical Skills Checklist

Use the checklist in Section 19 for instructor assessment and remediation.

26. References

[1] American Heart Association, CPR and ECC Guidelines, including 2025 guidance.

[2] International Liaison Committee on Resuscitation, publications and evidence-based resuscitation guidance.

[3] International Federation of Red Cross and Red Crescent Societies, International First Aid, Resuscitation and Education Guidelines 2025.

[4] American Heart Association and American Red Cross, 2024 Guidelines for First Aid.

[5] American Red Cross, first-aid steps and emergency response resources.

Source-use statement: The references provide recognised international guidance. They do not replace Nigerian emergency-service instructions, employer policy, approved instructor protocols, equipment manufacturer directions or applicable law. NIC must verify current versions and insert local emergency contacts before publication.

27. Quality assurance review

Quality question	Finding	Action before launch
Safety	Scene safety, emergency activation and second-casualty prevention are central.	Add local hazards and emergency contacts.
Current guidance	CPR parameters are linked to current recognised guidance and flagged for pre-launch verification.	Clinical/resuscitation educator must approve final technical content.
Scope	Online knowledge is clearly separated from practical	Require scope acknowledgement.

	competence and clinical qualification.	
Practicality	SAFE-ACT, action cards and scenarios support pressure-based decisions.	Pilot with working caregivers.
Scenario quality	Scenarios test prioritisation, safety and escalation rather than memorisation alone.	Review for local realism.
Nigerian relevance	Course recognises varied infrastructure, home/facility settings and backup routes.	Insert verified local pathways.
Practical competence	Two credential pathways and observed checklist are provided.	Approve instructor standards and equipment.
Documentation	Handover, incident, near-miss and objective recording are included.	Cross-link NIC documentation CPD.
Professional credibility	Certificate and badge wording prevents overclaiming.	Obtain NIC governance and legal review.
Non-duplication	Complements existing caregiver, medication, nutrition, dementia, palliative and documentation CPDs.	Cross-link relevant modules.

Final implementation checklist

The portal should follow **COURSE → MODULES → LESSONS → MODULE ASSESSMENTS → FINAL ASSESSMENT → CERTIFICATE**. It should present the online certificate separately from any practical endorsement. Before launch, NIC should insert current local emergency numbers, facility escalation routes, AED locations, first-aid equipment requirements, practical provider standards, instructor qualifications, child/infant protocol materials, privacy requirements and renewal rules.

The intended professional standard is:

I can recognise an emergency, keep myself and the client safe, activate appropriate help quickly, provide first aid and basic life support within my training, communicate clearly, and document and report what happened professionally.